

D391

Healthcare Ecosystems

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

Items

1.

A state with a large rural population and three medical schools votes to expand Medicaid, adding 300,000 enrollees, with no new clinicians and no change to payment rates. Its governor campaigned on the change for two years. What is the most likely short-term effect on the healthcare ecosystem?

- A. Coverage rises, but wait times and total spending pressure increase unless capacity or efficiency grows.
- B. Nothing changes because the medical schools will supply new doctors.
- C. Total spending falls because more people have insurance.
- D. Access, cost, and quality all improve together.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2.

A large retail chain with 60,000 employees and stores in 20 states chooses a health plan for its workers and pays most of the premium. The plan is run by a national insurer that processes the claims. In ecosystem terms, what is the retail chain?

- A. A provider.
- B. A purchaser (a type of funder).
- C. A payor.
- D. A regulator.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3. [TRANSFER — reversed stem / different variable]

A state board revokes the license of a pharmacy that repeatedly filled prescriptions incorrectly. Which ecosystem role is the board playing?

- A. Payor
- B. Regulator
- C. Producer
- D. Provider

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4.

A patient's plan has a \$1,000 deductible that she has not yet met this year and 20% coinsurance. She pays a \$450 monthly premium through her employer. An outpatient surgery has an allowed amount of \$6,000 and is below her out-of-pocket maximum. How much does she owe for the surgery?

- A. \$1,200
- B. \$2,450
- C. \$2,000
- D. \$4,000

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5. [TRANSFER — reversed stem / different variable]

A patient has a \$7,000 out-of-pocket maximum and has already paid \$6,700 in deductibles and coinsurance this year. A new hospital bill would require \$2,500 in coinsurance. How much does she pay?

- A. \$300
- B. \$0, because she is close to the maximum.
- C. \$2,200
- D. \$2,500

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6.

An uninsured man with chest pain walks into the emergency department of a nonprofit hospital that takes Medicare. The hospital recently opened a new cardiac wing and has a policy of collecting a deposit from self-pay patients. What does federal law require first?

- A. Collect the deposit per policy, then treat.
- B. Provide all his cardiac care free of charge.
- C. Transfer him to a public hospital because he has no insurance.
- D. A medical screening exam and stabilizing treatment before any questions about payment delay care.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7.

A 24-year-old graduate student with type 1 diabetes, who ran a marathon last spring, wants to stay on her mother's ACA-compliant employer plan. The plan's insurer is headquartered out of state. What is true?

- A. She can stay on the plan until 26, and her diabetes cannot be excluded or used to raise her rate.
- B. She must leave the plan because she is over 21.
- C. She must buy her own plan because the insurer is out of state.
- D. She can stay, but diabetes care can be excluded as a pre-existing condition.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8.

A patient has knee surgery at an in-network hospital with a surgeon from her plan's network. The anesthesiologist, whom she never chose or met before the day of surgery, is out of network and bills her an extra \$1,800 above the plan's payment. The hospital was rated four stars. What does she owe the anesthesiologist?

- A. The full \$1,800, since the anesthesiologist was out of network.
- B. Half the balance, split with the hospital.
- C. Only her normal in-network cost sharing; the \$1,800 balance bill is barred.
- D. Nothing at all, not even her usual cost sharing.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9.

A clinic serving 9,000 patients reports that 82% of its adult diabetic patients had an A1c test in the past year. The clinic added two nurse practitioners last year. In the Donabedian framework, what kind of measure is the testing rate?

- A. Access
- B. Process
- C. Structure
- D. Outcome

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10. [TRANSFER — reversed stem / different variable]

A hospital wants an outcome measure for its heart failure program. Which one qualifies?

- A.** Number of cardiologists on staff.
- B.** Percent of patients given discharge instructions.
- C.** Percent of patients weighed daily.
- D.** 30-day readmission rate for heart failure patients.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11.

A county health department compares two programs. Program A costs \$200,000 and gains 20 quality-adjusted life years (QALYs). Program B costs \$500,000 and gains 30 QALYs. Program B was designed by a well-known university. What is the incremental cost per QALY of choosing B over A?

- A.** \$10,000 per QALY
- B.** \$16,667 per QALY
- C.** \$50,000 per QALY
- D.** \$30,000 per QALY

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

12. [TRANSFER — reversed stem / different variable]

A new screening program costs \$150,000 more than the current one, and its incremental cost-effectiveness ratio is \$25,000 per QALY. How many additional QALYs does it produce?

- A.** 0.17
- B.** 3,750,000,000
- C.** 6
- D.** 25

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer A Concept 1

A state ~~with a large rural population and three medical schools~~ votes to expand Medicaid, adding 300,000 enrollees, with no new clinicians and no change to payment rates. ~~Its governor campaigned on the change for two years.~~ What is the most likely short-term effect on the healthcare ecosystem?

Target word: trade-off after expanding access **Decides:** More covered people, same supply of clinicians.

A	Correct	Expanding access without capacity strains cost and quality.
B	Camouflage	Medical schools take years to add clinicians; they don't settle the short-term effect.
C	Inverted operation	More covered people using care raises spending in the short term.
D	Absolute language	Ignores the trade-off; nothing in the stem adds capacity.

2. Answer B Concept 2

A large retail chain ~~with 60,000 employees and stores in 20 states~~ chooses a health plan for its workers and pays most of the premium. The plan is run by a national insurer that processes the claims. In ecosystem terms, what is the retail chain?

Target word: role of the employer **Decides:** It buys coverage but does not process claims.

A	Adjacent category	Providers deliver care.
B	Correct	Employers buy coverage for their workers; the insurer is the payor.
C	Confusion set	The insurer that processes claims is the payor.
D	Adjacent category	Regulators set and enforce rules; the employer does not.

3. Answer B Concept 2 • transfer

A state board revokes the license of a pharmacy that repeatedly filled prescriptions incorrectly. Which ecosystem role is the board playing?

Target word: role from action — reversed **Decides:** Licensing and enforcing rules.

A	Adjacent category	Payors process and pay claims.
B	Correct	Setting and enforcing rules, including licensing, is regulation.
C	Confusion set	Producers manufacture drugs and devices.
D	Inverted operation	The pharmacy is the provider; the board acts on it.

4. Answer C Concept 3

A patient's plan has a \$1,000 deductible that she has not yet met this year and 20% coinsurance. ~~She pays a \$450 monthly premium through her employer.~~ An outpatient surgery has an allowed amount of \$6,000 and is below her out-of-pocket maximum. How much does she owe for the surgery?

Target word: patient share of one bill **Decides:** Deductible first, then 20% of the remaining \$5,000.

A	Dropped element	20% of \$6,000 skips the deductible.
B	Camouflage	Adds a premium, which is not part of cost sharing for this bill.
C	Correct	\$1,000 deductible + 20% × \$5,000 = \$2,000.
D	Inverted operation	80% of \$5,000 is the plan's share after the deductible, not hers.

5. Answer A Concept 3 • transfer

A patient has a \$7,000 out-of-pocket maximum and has already paid \$6,700 in deductibles and coinsurance this year. A new hospital bill would require \$2,500 in coinsurance. How much does she pay?

Target word: solve with the cap binding **Decides:** Only \$300 remains before the out-of-pocket maximum.

A	Correct	\$7,000 – \$6,700 = \$300; the plan pays the rest.
B	Absolute language	She still owes the remaining \$300.
C	Inverted operation	Subtracts the \$300 remaining from the bill instead of capping at it.
D	Dropped element	Ignores the out-of-pocket maximum.

6. Answer D Concept 4

An uninsured man with chest pain walks into the emergency department of a nonprofit hospital that takes Medicare. ~~The hospital recently opened a new cardiac wing~~ and has a policy of collecting a deposit from self-pay patients. What does federal law require first?

Target word: EMTALA duty **Decides:** Emergency condition at a Medicare-participating ED.

A	Inverted operation	Payment cannot delay screening or stabilization.
B	Absolute language	EMTALA requires screening and stabilization, not free care; he can be billed later.
C	Adjacent category	Transfer for financial reasons before stabilization is exactly what EMTALA bans.
D	Correct	EMTALA requires screening and stabilization regardless of ability to pay.

7. Answer A Concept 5

A 24-year-old graduate student with type 1 diabetes, ~~who ran a marathon last spring~~, wants to stay on her mother's ACA-compliant employer plan. ~~The plan's insurer is headquartered out of state~~. What is true?

Target word: ACA dependent and pre-existing rules **Decides:** Under 26; pre-existing condition cannot be excluded.

A	Correct	ACA dependent coverage to 26 and pre-existing condition protections.
B	Wrong stem number	The age limit is 26, not 21.
C	Camouflage	Insurer location does not affect dependent eligibility.
D	Dropped element	Leaves out the pre-existing condition protection.

8. Answer C Concept 6

A patient has knee surgery at an in-network hospital with a surgeon from her plan's network. The anesthesiologist, whom she never chose or met before the day of surgery, is out of network and bills her an extra \$1,800 above the plan's payment. ~~The hospital was rated four stars~~. What does she owe the anesthesiologist?

Target word: balance billing protection **Decides:** Out-of-network ancillary clinician at an in-network facility.

A	Inverted operation	This is the surprise bill the law prohibits.
B	Adjacent category	No such split exists; the law removes the balance bill.
C	Correct	The No Surprises Act protects patients from this type of balance billing.
D	Absolute language	She still owes normal in-network deductible and coinsurance.

9. Answer B Concept 7

A clinic ~~serving 9,000 patients~~ reports that 82% of its adult diabetic patients had an A1c test in the past year. ~~The clinic added two nurse practitioners last year.~~ In the Donabedian framework, what kind of measure is the testing rate?

Target word: classify the measure **Decides:** A test being done is something done to the patient.

A	Adjacent category	Access is an aim of the ecosystem, not a Donabedian category.
B	Correct	Performing a test is a care process.
C	Camouflage	The added nurse practitioners are a structure measure, but they are not what is being classified.
D	Confusion set	An outcome would be A1c control or complications, not whether a test was done.

10. Answer D Concept 7 • transfer

A hospital wants an outcome measure for its heart failure program. Which one qualifies?

Target word: reverse: pick the outcome **Decides:** Outcome = the patient's health result.

A	Adjacent category	A structure measure.
B	Confusion set	A process measure.
C	Inverted operation	A process measure — an action taken by staff.
D	Correct	Readmission reflects the patient's result after care.

11. Answer D Concept 8

A county health department compares two programs. Program A costs \$200,000 and gains 20 quality-adjusted life years (QALYs). Program B costs \$500,000 and gains 30 QALYs. ~~Program B was designed by a well-known university.~~ What is the incremental cost per QALY of choosing B over A?

Target word: incremental cost-effectiveness ratio **Decides:** Extra cost \$300,000 over extra 10 QALYs.

A	Wrong stem number	\$200,000 / 20 is A's own average cost.
B	Inverted operation	\$500,000 / 30 is B's average cost, not the incremental cost.
C	Dropped element	\$500,000 / 10 drops A's cost from the numerator.
D	Correct	$(\$500,000 - \$200,000) / (30 - 20) = \$30,000.$

12. Answer C Concept 8 • transfer

A new screening program costs \$150,000 more than the current one, and its incremental cost-effectiveness ratio is \$25,000 per QALY. How many additional QALYs does it produce?

Target word: solve for the benefit instead of the ratio **Decides:** Extra QALYs = extra cost / ICER.

A	Inverted operation	\$25,000 / \$150,000 divides the wrong way.
B	Inverted operation	Multiplies cost by the ratio instead of dividing.
C	Correct	$\$150,000 / \$25,000 = 6 \text{ QALYs.}$
D	Wrong stem number	The ratio's dollar figure in thousands, not the QALY gain.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	4	
Wrong stem number	A figure copied from the prompt that is not the answer.	3	
Adjacent category	The right idea in the wrong bucket.	7	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	10	
Absolute language	Always, never, guarantee — where judgment applies.	4	
Confusion set	Every option is real; the test is exact role-matching.	4	
Camouflage	Answers a stem fact that was never the question.	4	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	The access-cost-quality triangle	Access, affordability, and quality pull against each other; a change that improves one usually puts pressure on at least one of the others unless efficiency improves.	Assuming expanding coverage automatically lowers cost and raises quality at once.	A policy that expands one aim with no mention of new funding or efficiency.
2	Four ecosystem roles	Regulators set and enforce rules; providers deliver care; producers make drugs, devices, and supplies; funders pay for care — payors process claims and purchasers (employers, government) buy coverage.	Calling an employer a provider or an insurer a regulator.	An organization described by what it does: licenses, treats, manufactures, pays, or buys coverage.
3	Patient cost sharing	The patient pays the deductible first, then coinsurance on the remaining allowed amount, capped by the out-of-pocket maximum; premiums do not count toward either.	Applying coinsurance to the whole bill or counting premiums toward the deductible.	A deductible, a coinsurance percentage, and an allowed amount in one stem.
4	EMTALA	Hospitals with emergency departments that take Medicare must screen and stabilize anyone with an emergency condition regardless of insurance or ability to pay.	Thinking EMTALA guarantees free care or covers all hospital services.	An uninsured person arriving at an emergency department.
5	ACA insurance market rules	ACA-compliant plans cannot deny coverage or charge more for pre-existing conditions, must cover essential health benefits, and allow dependents on a parent's plan until age 26.	Believing insurers can still exclude pre-existing conditions in marketplace plans.	A pre-existing condition, a 24-year-old dependent, or a benefit category.
6	Surprise billing protection	The No Surprises Act bars balance billing for emergency care and for certain out-of-network providers at in-network facilities; the patient owes only in-network cost sharing.	Assuming any out-of-network bill must be paid in full by the patient.	An out-of-network clinician at an in-network hospital, or an emergency visit.
7	Donabedian quality framework	Structure is the setting and resources, process is what is done to the patient, outcome is the result for the patient's health.	Calling a process measure, such as percent screened, an outcome.	A quality measure to classify: staff ratios, steps performed, or patient results.

#	Concept	Rule	Trap	Tell
8	Cost-effectiveness for decisions	Compare options by incremental cost per unit of health gained (such as cost per QALY): extra cost divided by extra benefit.	Dividing total cost by total benefit or choosing the cheapest option regardless of benefit.	Two programs, each with a cost and a health gain.

Course scope (public description): Healthcare Ecosystems examines how the aims and elements of the healthcare ecosystem can affect client and patient outcomes. The course explores the main aims of healthcare access, affordability, and quality and how regulators, providers, producers, and funders (such as payors or purchasers) support those aims. The course also examines insurance regulations and reimbursement procedures that affect healthcare access and affordability and decision-making processes that support affordable, quality care for clients and communities. There are no prerequisites for this course.